جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فايزه محمود احمد مسعود
المحافظة : الجيزه **الحي / المدينة :** مركز الجيزه **العنوان :** -
رقم البطاقة : 26206112200563 **رقم التليفون :** 01121757524 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 11-06-1962 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فايزه محمود احمد مسعود
National ID: 26206112200563

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

05/21/2026

MDT FORM

Name:	Masoud	MRN:	11267
Age:	64 Y	ID:	26206112200563
Menopausal Status:	post	Phone:	01228198078
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	new

Clinical Data:

BX: idc g3

ER - **PR** - **HER** 3+ **KI67**

Sonomammography:

LT LIQ and lower central malignant mass with ipsilateral level 1 pathological LN

Mets Workup:

hepatomegaly
no mets
bone scan requested

History:

clinically : a case of LT IBC with palpable large axillary LNs

Recommendations:

MDT Outcome & Plan

for neoadj TCHP then MRM

R L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
DR HANAN EZZAT	DR AHMED NAGI				

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: فايزة محمود احمد مسعود **MR. :** 11267	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

LT LIQ and lower central malignant mass with ipsilateral level 1 pathological LN ER PR -ve HER2 +3 for c1 Taxotere Carbo auc 5 + phesgo PLEASE ADD SOLUCORTIF

postm left breast cancer ct4d n2 m0 idc 2 er0/8 pr0/8 her23+ ki 60% on exam left breast edema induration redness left ax ln 2x2 for c2 taxotere 135 mg (10% dr) carbopl auc 5 and 1 proph neupogen and c2 phesgo (dr bec of skin hypersenstivity reaction) pt will bring report of echo

COMPLIAN OF FEVER DIAHREA SKIN ERUPTION NASUIA FOR LAB , P/A US

Pt is newly diagnosed with diabetes, pt is ER PR 0/8 + intolerable side effects (recurrent fever + diarrhea g3 + hyperglycemia + poor oral intake) > to be shifted to weekly protocol for c3w1 taxol 75 mg/2 carbo auc 2, labs accepted and vitally stable + for c3 phesgo

clinical pharmacy note : wt 93 ht 162 sa 2 for c3w1 taxol 150 mg carbo auc 2

pt had multiple abscesses ,grade 1 pn for 15 % dr weekly tc is well tolerated for w2 taxol125 mg carbopl auc 2 on exam left breast edema peau d orange redness

for c3w3 taxol carboplatin, g1 PN + supportive tx, pt st hypoglycemic ttt for c4 phesgo + c4w1 w/labs + w2 phesgo

The patient has completed 3 weeks of taxol in cycle 3 and 4 weeks of phesgo.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

Presidential Initiative for Women's Health

Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

مستشفيات جامعة القاهرة

PATHOLOGY REPORT

Name: فايزه محمود احمد مسعود
Patient's I.D.: 26206112200563
Age: 63 Y

Pathology Number: 105 /2026
Specimen Received Data: 19 / 04 /2026
Date Report Issued: 26/ 04 /2026

Clinical data: Left breast LIQ mass, BIRADs 5.

Nature of the specimen: U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of breast tissue, showing an invasive malignant tumor formed of groups, tubules & cords of moderately to markedly anaplastic epithelial cells. Surrounding desmoplastic stroma with 5-10% TILs was seen.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:
Examination of section treated for estrogen receptors revealed moderate positive nuclear staining in <1% tumor cells by chromogen (Score 3/8).
Examination of section treated for progesterone receptors revealed negative nuclear staining of tumor cells by chromogen (Score 0/8).
Examination of section treated for Her-2/neu revealed strong complete membrane staining in >10% of tumor cells by chromogen (score 3+).
Examination of section treated for Ki67 revealed positive nuclear stain in average of ±60% of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy:

Invasive Duct Carcinoma, NST, Grade III.

- ***Negative*** **For Estrogen Receptors.**
- ***Negative*** **For Progesterone Receptors.**
- ***Positive*** **For Her-2/neu (Score +3).**
- ***60%*** **Ki-67 Proliferative index**

PROF. DR. SOMIA ABDEL LATIF
DR. PASSANT ESSAM SHIBEL

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

١١٢٦٧

Patient Name: FAIZA MAHMOUD AHMED MASOUD	Patient ID: 26206112200563
Study Date: 19/04/2026	Age: 64 Years

Bilateral Full Field Mammography with Complementary Ultrasound Examinations Revealed

Indication:

- *Left breast mass.*

Findings:

- *Heterogenous fibroglandular parenchyma (ACR C).*
- *No right spiculated masses or microcalcific clusters.*

Left breast:

- *Lower inner and lower central irregular hyperdense lesion is seen with overlying few faint microcalcifications associated with increased skin thickness of 5 mm and diffuse parenchymal and interstitial edema, ultrasound revealed left 6- 7 o'clock irregular hypoechoic lesion measuring 5.2x4 cm with abnormal internal vascularity, it measures about 3mm from the skin and about 1.5 cm from the nipple (measured in the CC view). Left UOQ confluence of ducts/complicated cyst is seen measuring about 9x5mm. Mild dilatation of the subdermal lymphatics is also noted.*
- *Bilateral echogenic parenchyma with scattered variable cysts, some of them are complicated.*
- *Multiple pathologically enlarged left axillary level I lymph nodes showing effaced fatty hila, largest measures 2.5x3.6 cm.*
- *No pathologically enlarged supraclavicular lymph nodes.*

Right breast

- *Bilateral echogenic parenchyma with scattered variable cysts, some of them are complicated.*
- *Normal skin thickness and contour.*
- *Normal nipple areola complex.*
- *Non specific axillary lymph nodes.*

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name: FAIZA MAHMOUD AHMED MASOUD	Patient ID: 26206112200563
Study Date: 19/04/2026	Age: 64 Years

Opinion:

- *Left LIQ and lower central malignant looking mass lesion with ipsilateral axillary level I pathological lymph nodes, for histopathological assessment and better contras enhanced study. (BIRADS 5).*
- *Fibrocystic disease of both breasts (BIRADS 2).*

Thanks for reference,

Dr. Shereen Abdelraouf, MD

Ass. Prof. Sherihan Fakhry.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name: FAIZA MAHMOUD AHMED	Date: 12/05/2026
Age:	ID: 100408707

V-1V69

GIT IMAGING UNIT

POST CONTRAST CT EXAMINATION OF ABDOMEN & PE

REVEALED:-

**Reported history of breast cancer, for metastatic workup.

- Enlarged liver showing smooth outline and homogenous parenchymal texture with no definite hepatic focal lesions. No dilated intra hepatic biliary radicles.
- Distended gall bladder showing few hyperdense stones with no pericholecystic edema or fluid collections.
- Retro-aortic left renal vein (Variant).
- Small uterine calcifications are seen.
- Non complicated colonic diverticulae.
- Underfilled urinary bladder with no sizable focal masses.
- Normal CT appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta (apart from small non significant aorto-iliac atheromatous calcifications), and rest of the pelvic organs.
- Subcentimetric para-aortic lymph nodes. Otherwise, no CT evidence of lymphadenopathy or pathological masses.
- Small hiatus hernia.
- No ascites or pelvic collection.
- Scanned bone window shows
  - Dorsolumbar spondylosis with first degree degenerative spondylolisthesis of vertebra.
  - Left iliac and smaller right femoral head sclerotic foci, possibly bone islnads, for bone scan correlation.
  - Bilateral sacroiliitis.

OPINION:-

- Hepatomegaly with no definite hepatic metastatic deposits.
- Calcular gall bladder.

Dr. Marwa Shaker MD

Dr.Nouran M.Abdallah,

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient name: FAIZA MAHMOUD AHMED	Age:
Modality Name: CT	Study Date: 12/05/2026 12:12:42
Patient id: 100408707	

THORACIC IMAGING UNIT

POST CONTRAST CT SCAN OF THE CHEST REVEALED:-

- Reported case of breast cancer showing:
- Left breast ill-defined isodense mass with cystic areas of breaking down measuring roughly about 4x4.6cm associated with overlying skin thickening and multiple enlarged axillary lymph nodes at level I and II , largest measuring about 2.3cm in its short axis.
- Right lower lobar osteophyte related reticulations.
- Lingula thick parenchymal bands.
- no evidence of sizable nodules, cavitations or masses.
- No pleural or pericardial sac collections.
- No enlarged hilar or mediastinal lymph nodes.
- Small sliding hiatus hernia
- Dorsal spondylosis.
- CT signs of cardiomegaly.

OPINION:-

- Left breast ill-defined mass with cystic areas of breaking down associated with overlying skin thickening and multiple enlarged axillary lymph nodes
- Cardiomegaly for echo correlation

Dr.Nourhan Mohamed,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Faiza Mahmoud Ahmed Masoud
Branch : Maadi
Ref.Physician Prof : Goustav Roussey Egypt

Date : 20-06-2026 12:31
Medical No. : 398112
Ins.Company : تعاقد مستشفيات

Whole-Body Bone Scintigraphy

Clinical Indication

- Staging/follow-up evaluation for a 64-year-old female patient with a history of left breast carcinoma, presenting with generalized skeletal pain.

Procedure

- Whole-body anterior and posterior planar imaging acquired 3 hours following the intravenous administration of Tc-99m MDP.

Findings

Axial Skeleton:

- Spine: Heterogeneous radiotracer accumulation localized to the lower lumbar vertebrae, consistent with exaggerated degenerative changes. The cervical and thoracic segments show normal tracer distribution.
- Thoracic Cage, Skull & Pelvis: Normal, symmetric radiotracer uptake throughout the skull, ribs, sternum, scapulae, and pelvic girdle. No focal osteoblastic lesions detected.

Appendicular Skeleton:

- Joints: Mild-to-moderate, bilaterally symmetric increased tracer uptake involving the knees and ankles, compatible with osteoarthritis.
- Extremities: Normal, symmetric cortical tracer distribution throughout the upper and lower long bones.

Soft Tissue & Excretion:

- Normal renal parenchymal activity and physiological urinary bladder clearance.

Conclusion

- No scintigraphic evidence of active osteoblastic metastatic disease.

Dr. Alaa Nagi, MD
Nuclear Medicine

PET Scan
[[DOCTYPE:PET_SCAN]]